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FCF|REACT Cash & Livelihoods Monitoring and Evaluation

Post-Distribution Monitoring 1 Report

USAID ជំនួយពីពលរដ្ឋអាមេរិក

GHR FOUNDATION

Save the Children

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DISCLAIMER

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1. Executive Summary

This report is a follow-up study related to post-distribution monitoring (hereafter referred to as PDM1) of the cash transfer component of the program from the previous baseline(+PDM) conducted. It is important to note that between the data collection undertaken for the baseline(+PDM) and this PDM1 study there has been a deterioration of the COVID-19 situation in Cambodia, with nationwide school closures, movement restrictions and lockdowns. Considering the evolving context is very likely to exacerbate existing vulnerabilities and create new ones, the respondents maintaining their socio-economic situation should be considered as a positive development. The results of this PDM1 - which had a scope limited to the use of the cash transfers and their safe and equitable distribution - continues to be very positive and in many regards improves upon the results of the previous baseline(+PDM) study.

100% of respondents now report that the cash is the most effective form of assistance for the family to meet their basic needs, up from 96% in the baseline(+PDM) study and 11% of the respondents reported that they can fulfil all their basic needs, up from 4%. The number of respondents who reported concerns about accessing the money was reduced with 100% reporting that they felt safe while receiving the money and no one reporting having heard of anyone else experiencing an unsafe situation. In total, 100% of the respondents reported that the cash transfers had a positive effect on their household or relationship within them, up from 80% in the baseline(+PDM). Furthermore, no negative effects of cash transfers were mentioned. In other results, 82% of women reported shared decision-making on cash transfer use, which remains static compared to baseline(+PDM).

One of the key improvements suggested in the previous study was regarding notifications to beneficiaries about why they were selected for cash transfers, the amount of money they receive and when they are to receive it, and in this study 87% of respondents achieve all three points, up from 54% in the baseline(+PDM). However, another suggestion of the baseline(+PDM) to improve knowledge of methods of filing complaints has not improved as only 33% of households could identify at least one available channel for providing complaints or feedback about the assistance, slightly lower than the 40% achieved in the baseline(+PDM).

Although the overall results are very positive some concerns remain. Spending on food has increased in importance from the previous study's already very high levels and reports of increased food prices compound issues – although SCI is monitoring this and with support from the WFP, as of 17th May 2021, report no consolidated evidence of major food price increases within the country. Evidence would suggest that children are having issues in getting education and there is an indication that children might be going to work more than before. Key conclusions and recommendations found from this study are:

1. More households reported sharing their cash with people outside of their households.

From the results of this study, it is unclear why these respondents are sharing the money with others. Due to the unprecedented times, it may be that they are helping others in their family or community who are in desperate situations. Unfortunately, at the moment we do not know why they are sharing with others and possible implications of this. It is recommended that case workers follow up with these respondents or that a question is added to the next set of data collection to understand this further.

2. Greater understanding on the effects of COVID-19 on education is needed.

Although it is not asked directly as part of the PDM1 data collection, it stands to reason based on the national closure of schools and decreased number of respondents mentioning expenditure on education that children's education may have been negatively affected by the latest outbreak of COVID-19. It was outside of the scope of this PDM1 study, but it would be interesting to understand further about the effects of COVID-19 on the children's education. In the PDM2 study to be conducted questions regarding attendance are covered (as they were in the baseline(+PDM), yet, more qualitative understanding regarding whether families are capable of enabling remote learning for their children, if they organise any alternative learning and how effective any of this different way of learning is, would

be interesting to understand. Understanding could possibly be achieved through a selection of additional questions added to PDM2, social workers gathering information through case management/money matter sessions or as part of other SCI or partner projects.

3. **Understanding effects of current COVID-19 community outbreak on children entering the workforce.**

The number of respondents that mentioned that a positive effect of the cash transfers is that children didn't need to go to work reduced from 41% to 14%. This is not clear evidence that their children have in fact started working, but with school closures, it is a concern. The PDM2 study asks questions directly related to children's work and will provide clearer evidence if this is, or is not, occurring. However, in the meantime, it would be proactive for social workers to begin discussing this with families. As such, SCI and partner organisations should raise awareness to social workers of this possible issue.

4. **Respondents have increased paying off debts.**

Many more households have reported paying off debts than in the baseline(+PDM) previously. Currently, there is little evidence to point to the underlying reasons as to why debts are being paid off and so the rationale may be either positive (e.g., increased income) or negative (e.g., coercion from debt collectors). As the reasoning may be sensitive and as it is difficult to ask succinctly within a questionnaire, it is recommended that further understanding of this topic is not added to the PDM2. Instead, it is recommended that case workers are familiarized with this issue and may follow up with beneficiaries during their next visitation.

2. Context

2.1 The Child-Parent Relationship in Cambodia

(A family receiving Child Seperation Prevention during COVID-19 pandemic, CCT 2020)

Cambodia has made great improvements concerning children's health, education, and their protection, strongly driven by Cambodia's impressive economic growth during 1995 to 2017, when Cambodia was

the sixth fastest-growing economy in the world and the poverty rate fell from 47.8% to 13.5%. Labor force participation increased to approximately 84 per cent, around 80 per cent for women and around 89 per cent for men in 2017 (National Institute of Statistics, 2018). As a result, Cambodia's economy has shifted from a low, to middle-to-low, income country, with the ambition of becoming a middle-income country by 2030.

Despite these nationwide accomplishments, a complex array of factors still prevent many children from reaching their full potential. Cambodia's population is still roughly 90% rural, and multidimensional poverty remains high with the expectation of the poverty rising to 17.6% due to the Covid-19 pandemic (The World Bank, 2020; UNDP, 2020).

The engagement of both parents in making an income has posed challenges regarding the care of children by their parents. Children, suggests Murage (2019), do not have much time with their parents because they are busy with their work, and many children do not see their parents until the weekend. Cambodian children aged below 15 years are often left behind with grandparents, hired caregivers or other relatives, instead of being with their working parents (Zimmer and Natta, 2015; Laidler, 2018). The absence of parents in caring for children at home due to their work, or migration for work, can lead children to take on the care-giving roles in the family with older children often responsible for taking care of younger siblings when their parents are occupied with their business (Murage, 2019) and sometimes children are forced to drop out of school to help with the household chores, caring work or other needs to support the family (Laidler, 2018).

In poorer families, parents or caregivers sometimes send their children to Residential Care Institutions (RCIs) to obtain care, food and education for their children (Csáky, 2009). RCI's have been proven to be more costly than family-based care and can have a number of harmful impacts on a child's development. Some RCIs maintain relationships with the children's' parents, but others have been known to prevent a child's contact with their parents. RCI's have been known to send children to other, further locations without their parents' consent (Better Volunteering Better Care, 2014). Furthermore, some RCIs in Cambodia have been involved in child trafficking and child abuse (Farley, 2015; (Murdoch, 2015).

2.2 Policy

Due to the increase in the number of children being placed in RCIs, the Cambodian government issued a Policy Paper on 'Alternative Care for Children in Need of Special Protection' and 'Children at Risk' in 2006 (MoSVY, 2006). Principles in the policy paper include:

- Residential care should be a last resort and a temporary solution for children.
- Family-based care and community-based care are the best choices for alternative care.
- Specific strategies and measures shall be developed to support parents to raise their children and send them to school.
- These strategies and measures shall also be directed to families, relatives and communities caring for children where parental care is not possible, to avoid institutionalization.
- Where residential care cannot be avoided, minimum standards and guidelines for residential care are to be implemented, monitored and evaluated in accordance with the overriding principles of the 'Convention on the Rights of the Child.'
- Residential care and/or community or family-based care are requested to strive to meet minimum standards developed within the framework of this Policy. NGOs which provide alternative care service must request permission from and sign an agreement with the Ministry of Social Affair, Veterans and Youth Rehabilitation (MoSVY).

The Sub-decree on the Management of Residential Care Institutions was developed and came into effect in September 2015 intending to put in place 'gatekeeping' mechanisms to reduce the intake of children into residential institutions (MoSVY, 2015).

2.3 Program Background

FCF|REACT is a network of organizations working together to support children to live in safe, nurturing, family-based care facilitated by Save the Children (SCI). SCI works in collaboration with the government, local and international NGOs, academic institutions and UN agencies, to enhance and strengthen family-based care. SCI, together with around 60 member organizations, are working to prevent children from being separated from their families and increase the number of children who are safely and successfully integrated into family-based care. The intervention is done to strengthen systems and policies and to provide services to children and families directly.

(A family receiving support during COVID-19 pandemic, CCT 2020)

The COVID-19 pandemic threatens children and their rights and exposes them to potentially significant disruption to their healthcare, education, access to basic needs and services such as food, protection and social interaction. Loss of income and inflated prices will increase food insecurity among the most vulnerable families, intensifying the risks of family separation as poverty is recognized as a significant factor in the placement of children in RCIs.

By implementing child-sensitive livelihoods to secure/restore basic incomes and providing child-sensitive cash transfer assistance, SCI aim to enable vulnerable families to access sufficient resources to invest adequately in their children, to help children and families to stay together safely, and mitigate the economic impact of COVID-19 in the medium term.

2.4 Changes in country context since the Baseline (+PDM) study

Since the baseline (+PDM) study a lot has changed in Cambodia with regards to the COVID-19 situation. Below is an outline of government policy which have occurred since that study which may have affected some of the results of this project and continue to affect the lives of the beneficiaries involved.

- Schools within Phnom Penh and Kandal were closed on the 22nd of February. Nationwide schools were closed on the 21st of March and in all cases have yet to re-open, although online and television education is available, it is likely not accessed by all.
- Some zones in Phnom Penh identified as having high numbers of COVID-19 cases began lockdown from 6 April.

- Inter-provincial travel was banned for 14 days from 7 April but continued in Phnom Penh and Kandal for longer.
- Official lockdown was imposed in Phnom Penh and Takmao from 15 April and was extended until 5 May.
- Siem Reap announced travel restriction and commune lockdown in three communes from 16 April, 2021.
- In Phnom Penh, Kandal and some other provinces a 3-color system was established (red, orange, yellow) relating to the severity of restrictions within these locations. In red zones especially, access to food was made more difficult as movement was limited and anecdotal reports of cost of food increases appeared.
- At the time of writing some restrictions remain.

Furthermore, it is worth noting that although the draft baseline(+PDM) report had been shared with partners prior the fieldwork for this PDM1 study, the final report and structured presentation and discussion with partners on how to respond to recommendation did not occur until after the completion of field work for PDM1.

3. Objectives

3.1 Objectives of FCF|REACT Cash & Livelihoods

(A family getting introduced and oriented about the cash transfer program, CCT 2021)

The FCF|REACT 'Cash & Livelihoods' program, with support from USAID, was created with the aim to improve the capacities of vulnerable families affected by the secondary impacts of COVID-19, to stay together in a caring and nurturing environment and to reduce protection risks for their children through two components:

- Child Sensitive Cash transfers assistance to support households in situations of high economic insecurity due to the impacts of COVID-19 to prevent them from resorting to negative coping strategies that could lead to increased child protection risks.
- Child sensitive livelihood promotion interventions, including training and micro-grants provision for vulnerable households and youth who have lost their sources of income due to COVID-19, to prompt livelihoods restoration and prevent the use of negative coping strategies potentially harmful for children, in a sustainable manner.

3.2 Objectives of the PDM1 study

The objectives of the PDM1 study are to:

- Undertake post-distribution monitoring for the cash component of the program to understand how the cash transfers are being used by households, the outcomes it is having in households, and to identify any unintended safety and equity issues in the distribution.
- Create comparisons versus the Baseline(+PDM) evaluation to understand progress and any changes among the beneficiaries.
- Generate strategic evidence to contribute to the FCF|REACT Learning Agenda

4. Methodology

4.1 Methods and Sample Size

As with the Baseline(+PDM) before it, this project, PDM1, relies on a primarily quantitative framework with fieldwork undertaken with around 10% of project beneficiaries (N=70). Due to the sensitive nature of the project, interviews were to be undertaken face-to-face with the respondents in each location, following COVID-19 safe guidelines. However, due to the community transmission of COVID-19 in Phnom Penh and Kandal, there were lockdown restrictions in place which did not allow face-to-face meetings. As such, telephone interviews were conducted with the n=4 respondents in these locations. Additional precautions were taken during these four interviews to ensure that the respondents were alone and could answer the questions both safely and accurately. These precautions included:

- Enumerators included a revision in their introduction to respondents that suggested taking the call at a time and place where they felt comfortable to talk freely and comfortably. This was mentioned both in the initial call to set a time and reiterated at the start of the actual interview call.
- Enumerators asking the questions included a revision prior to violence-related questions where it is reiterated that if the respondents are uncomfortable to answer they can just say so.

Besides the slight revisions, questions asked within the PDM1 study were all included within the Baseline(+PDM) conducted in March. The interviewers used for this data collection were the same as those used previously and as such training conducted prior to fieldwork was shortened to a one-day refresher training session conducted remotely over Zoom.

As in the baseline(+PDM), fieldwork was prioritized within the four major locations of beneficiaries including Phnom Penh, Kandal, Siem Reap and Battambang. 34 households from Battambang, 32 from Siem Reap, 2 from Phnom Penh and 2 from Kandal were included in the study.

To ensure comparability of data, respondents were selected based on their 'score' in the previously conducted Household Economic Survey (HES), which was used to identify households (among those already receiving case management services from partners) that met a scoring threshold to be considered 'eligible' for cash transfers. This score is a calculation of vulnerability based on household composition and socio-demographic factors, economic vulnerability, food insecurity and child protection risks.

The sample below was selected before fieldwork started. Although efforts were made to create the pools as homogeneously as possible, there are some differences among key criteria between the waves of data collection. The column for the Baseline(+PDM) and for PDM1 data in the table below is the actual result. For the other stages of fieldwork, it is the planned demographics of the respondents to be selected. Therefore, the actual demographics of the respondents interviewed in each wave may change. Differences between the demographics of the respondents among each wave will be incorporated into the analysis and noted in each report.

	Baseline (+PDM)		PDM1		PDM2		Endline		All Eligible HH	
TOTAL HES Score										
30-39	14	20%	13	19%	14	20%	14	20%	130	20%
40-49	19	27%	22	31%	20	29%	20	29%	189	29%
50-59	24	34%	19	27%	19	27%	19	27%	170	26%
60-69	8	11%	13	19%	11	16%	11	16%	111	17%
70+	5	7%	3	4%	6	9%	6	9%	59	9%
Dependency Ratio										
Less than 1.5	36	51%	27	39%	31	44%	30	43%	293	45%
Dependency Ratio (1.5 to 2)	20	29%	23	33%	19	27%	21	30%	202	31%
Dependency Ratio (2.1 to 3)	8	11%	13	19%	12	17%	10	14%	91	14%
Dependency Ratio (more than 3)	6	9%	7	10%	8	11%	9	13%	72	11%
HH w/ Disabled/Chronically Ill										
N/A	20	29%	18	26%	32	46%	27	39%	222	34%
1 person	25	36%	28	40%	20	29%	21	30%	209	32%
More than 1 person	25	36%	24	34%	18	26%	22	31%	222	34%
ID Poor Status										
No	17	24%	17	24%	29	41%	28	40%	227	35%
Type 1	20	29%	23	33%	18	26%	26	37%	219	34%
Type 2	33	47%	30	43%	23	33%	16	23%	206	32%
Indebtedness										

None	16	23%	13	19%	9	13%	29	41%	163	25%
<USD500	20	29%	31	44%	27	39%	20	29%	235	36%
USD500+	34	49%	26	37%	34	49%	21	30%	254	39%
Extent of Income Decrease										
Partly	17	24%	14	20%	20	29%	11	16%	153	23%
Majority	37	53%	33	47%	31	44%	38	54%	325	50%
All	10	14%	15	21%	13	19%	14	20%	122	19%
None	6	9%	8	11%	6	9%	7	10%	52	8%
Household Hunger Scale										
Little to no Hunger	35	50%	28	40%	25	36%	32	46%	306	47%
Moderate	21	30%	28	40%	21	30%	16	23%	183	28%
Severe	14	20%	14	20%	24	34%	22	31%	156	24%
rCSI Score										
<4	3	4%	0	0%	1	1%	1	1%	13	2%
4-19	11	16%	15	21%	15	21%	13	19%	111	17%
19+	56	80%	55	79%	54	77%	56	80%	528	81%
Adopted Harmful Coping Strategies										
No	14	20%	18	26%	16	23%	14	20%	137	21%
One	27	39%	24	34%	21	30%	26	37%	241	37%
More than one	29	41%	28	40%	33	47%	30	43%	274	42%
Likelihood of Separation										
Unlikely	29	41%	26	37%	14	20%	24	34%	196	30%
Likely	17	24%	21	30%	31	44%	24	34%	215	33%
Highly Likely	6	9%	9	13%	9	13%	6	9%	85	13%
Happened	18	26%	14	20%	16	23%	16	23%	163	25%

Green indicates a percentage that is at least 5% higher than the average from all eligible households.

Orange indicates a percentage that is at least 5% lower than the average from all eligible households.

4.2 PDM1 Data Collection

To undertake the quantitative data collection, the following steps were taken:

1. Development of research tools

The research tools used in this project were created by SCI and utilized a number of existing tools to gather information on the various project indicators. The tools were submitted to the Save the Children US Ethics Review Committee for approval (SCUS-ERC-FY2021-25).

2. Questionnaire translation by the Spear Insights team.

The survey was translated by one member of staff before being edited by a second and a third member. After the third edition, the team met to finalize the translation.

3. Confirmation of the questionnaire by SCI.

Upon completion of the final draft by Spear Insights the translations were sent to SCI for their input or possible suggestions, in particular regarding the correct use of technical terms, before being returned to Spear Insights to finalize, for programming.

4. Programming of the questionnaire into the KoBoToolbox survey tool.

KoBoToolbox was chosen as it was simple to use for enumerators and allowed data collection through offline mobile devices while in the provinces. The questionnaire was programmed in both English and Khmer as, although it was administered in Khmer, some testing of the programming and analysis of the final results were completed in English.

5. Testing and internal pilot tests of the software.

Usability of the survey, skip logic and typos were tested during internal testing of the software.

6. Confirmation of the data collection software by SCI.

An online link to the survey was shared with SCI to test and confirm the software programming.

7. One-day refresher fieldwork training

Refresher training occurred over one day on the 12th April 2021 to discuss the changes in the questionnaire since the Baseline(+PDM) and to ensure that each enumerator had a clear understanding of the project's objectives, respondent sampling, the format and flow of the questionnaire, ethical concerns and expectations, quality assurance and data monitoring. The training took place under the following guidelines.

a. 12th April Morning: Refresher on the background and overview of the 'Cash Transfers and Livelihood' program, explanation of the sampling methods and ethical concerns for the PDM1. A guided read-through of the questionnaire was conducted.

b. 12th April Afternoon: Additional read-throughs of the questionnaire using the KoboToolbox program. Q&A with the enumerators.

8. Data collection was undertaken by the field teams in each province.

The fieldwork team consisted of three enumerators; one in Siem Reap, one in Battambang and one in Phnom Penh/Kandal. Data collection was undertaken 17- 27 April 2021.

9. Data monitoring

During the completion of fieldwork, enumerators called the fieldwork manager in Phnom Penh to check in and discuss any issues, challenges or concerns they had found regarding the fieldwork. These issues included dealing with the unpredictable COVID-19 outbreaks, uncontactable/unresponsive respondents, changes in sampling or suggestions to re-order samples due to uncontactable samples. The team created a telegram group including both the fieldwork team and the team at the head office in Phnom Penh to ensure immediate support for any issues. In addition, the team at the head office in Phnom Penh monitored the data daily, checking key indicators such as interview length, logical responses and the quality of open answers.

10. Analysis of results upon completion of fieldwork.

Data was exported into excel from KoBoToolbox and then imported into SPSS for cross-tabulation of response criteria and data analysis.

4.3 Data Analysis and Reporting Plans

After completion of data collection, the quantitative data was exported from KoBoToolbox into excel, formatted and then imported into SPSS for analysis. In SPSS, the responses to questions and indicators

were cross-tabulated against key respondent identifiers such as gender, disability, IDPoor status level, Household Hunger Scale (HHS), Reduce Coping Strategies Index (rCSI) score and likelihood of separation. Usual demographic measurements such as location are unlikely to be used as the project is being analyzed at a project level with a representative sample and as such, there are low numbers of respondents in some locations. The project indicators to be analyzed in each wave of the project are outlined in section '7.1 Evaluation Framework' of this report according to the indicator definitions/analysis provided by SCI in the project MEAL Plan.

As mentioned in the sample size section earlier, so that the results are comparable, the sample has been selected so that there are minimal differences in the make-up of respondents between waves of fieldwork. Significance of results may be difficult to achieve as the sample sizes used in this research are small. With that noted, significance tests will be applied where possible and any significance in changes will be remarked upon if found.

In addition, many of the sampled households are also IDPoor and as such receive additional supports compared to non-IDPoor households (although some analysis will be made to compare those within and outside of the IDPoor scheme).

4.4 Ethical Considerations

Before undertaking data collection each of the project partners reached out to the selected respondents and asked for their informed consent to participate in the process and have their contact details shared with Spear Insights. Once the agreement had been confirmed, the Spear Insights team called each of the respondents to schedule their interviews so that they agreed and were aware of the time to expect the enumerators, before their arrival.

(A partner staff practicing the household economic survey with a beneficiary, FCF|REACT, 2020)

Once in the field, the enumerators asked for consent from the respondent during the introduction and the opening question of the survey. The informed consent form contains three main elements including

the purpose of the evaluation/survey, the rights to voluntarily participate and/or withdraw from participating and the confidentiality of answers, with the exception of any harm to a child being identified. The consent form was read out loud by the data collectors to the participants to ensure they understood what was written in the form, before they voluntarily agreed or not, to be interviewed.

All evaluation team members had adequate experience in collecting sensitive information, specifically regarding information concerning violence against women and children. Moreover, the Child Safeguarding policy was introduced to all consultants/enumerators and everyone had to sign the Child Safeguarding policy to ensure that they understood their responsibilities under the policy to protect children from any harm during the evaluation process.

The interviews conducted face to face were undertaken in open spaces in response to the respondent's convenience and COVID-19 safety as well. Moreover, consideration of the environment, ensuring that it was free from any disturbance by family members or neighbors was considered by the enumerators, to ensure the confidentiality of the respondents. For the interviews conducted by telephone, the respondents were asked before starting if they were in a safe and disturbance free space to be interviewed. Financial incentives were not provided to respondents within the evaluation process.

In the event that the enumerators became aware of any harmful situation occurring or becoming apparent during the fieldwork, a process had been put in place so that they would report to Spear Insights' management immediately who, in turn, would immediately inform SCI for the relevant referral or intervention. If further assistance was needed by the respondents, in terms of social or psychosocial support, the evaluation team would provide the information to SCI, who in turn would inform the partner/caseworker responsible for the household in question, to provide further support. However, during the PDM1 data collection, there was no situation identified which required reporting.

The research tools were submitted to the Save the Children US Ethics Review Committee for approval (SCUS-ERC-FY2021-25).

5. Findings

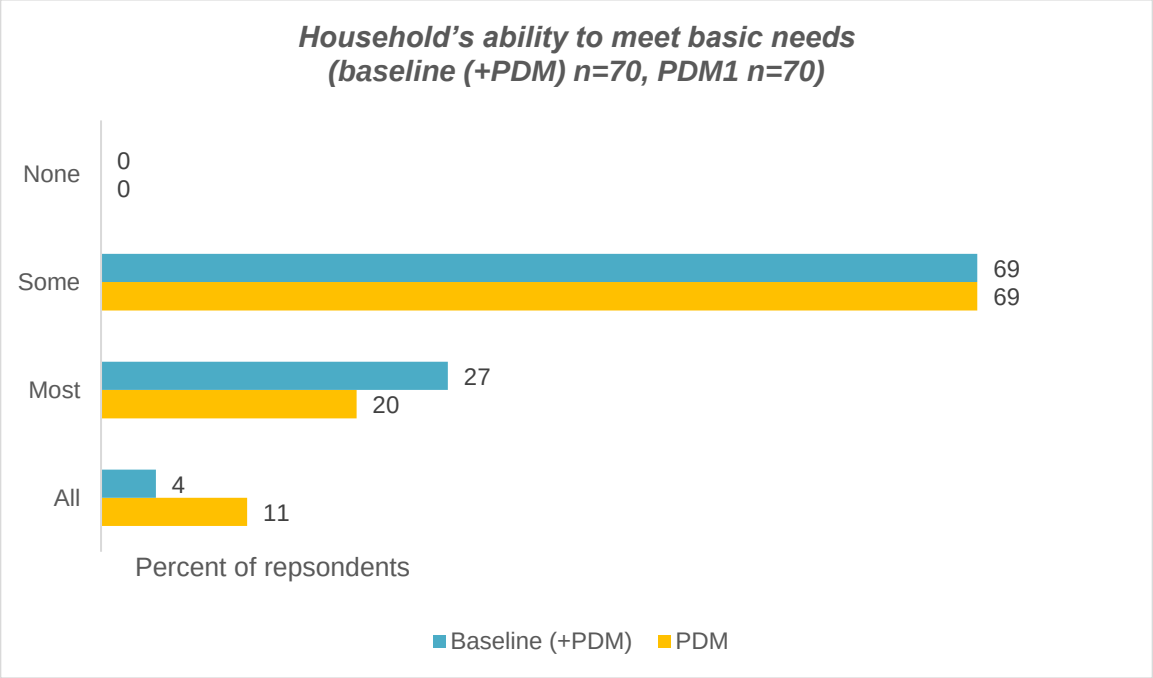
(A household receiving their cash, CCT 2021)

5.1. Percentage of households who report being able to meet the basic needs of their households (all/most/some/none) according to their priorities

Key indicator insight:

The concept of basic needs refers to the essential goods, utilities, services or resources required on a regular or seasonal basis by households for ensuring long term survival AND minimum living standards, without resorting to negative coping mechanisms or compromising their health, dignity and essential livelihood assets. This indicator attempts to measure how well beneficiary households are able to meet their basic needs (as they define the term).

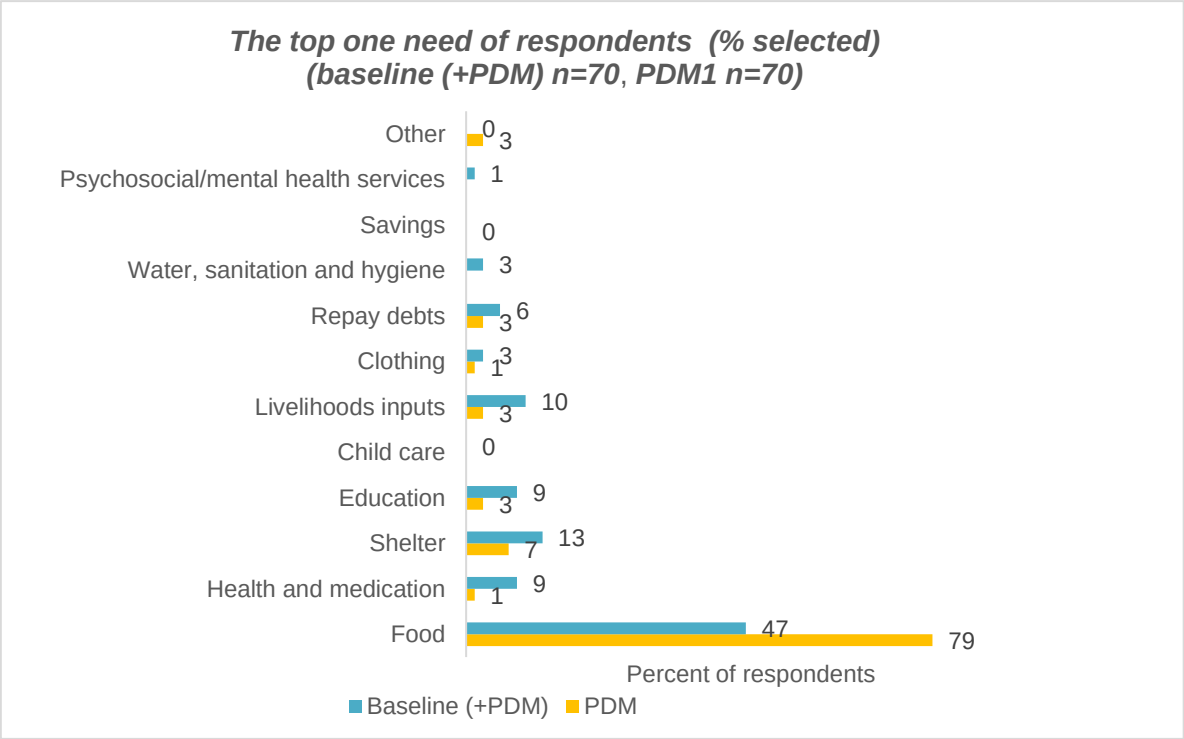
Amidst all households, 11% (8/70) of them claimed that they can fulfil all their basic needs, followed by 20% (14/70) who can fulfil most of their basic needs whilst most (69% - 48/70) can still only fulfil some of them. Compared to the baseline(+PDM) study those who can fulfil all of their needs increased by 7 percentage points whilst those who can fulfil most decreased by 7 percentage points. None of the respondents claimed that they were unable to fulfil any of their basic needs which is positive. All respondents (70/70) cited the cash assistance as being the most effective method of helping them and their families to meet their basic needs, compared to 96% (67/70) in the baseline(+PDM), therefore the usefulness of the cash transfers remains high.

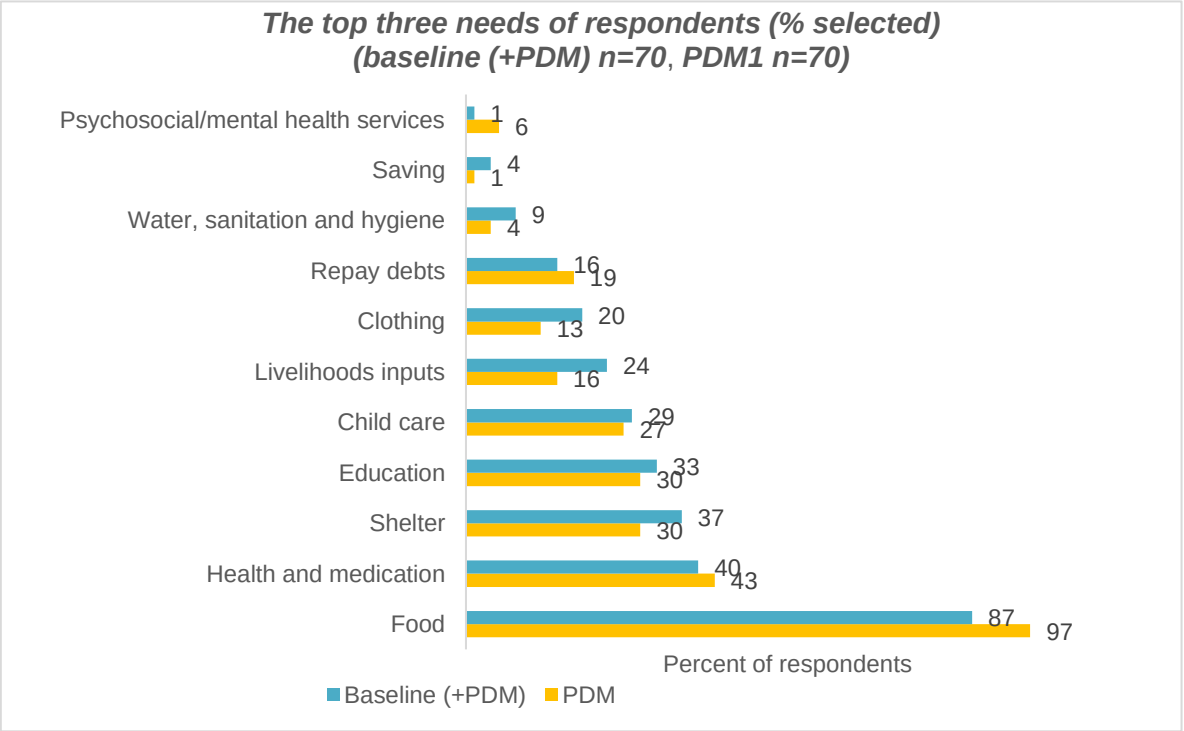


Each respondent was asked to rank the top three priority needs of their household. Food was still ranked as the most important item but it had increased almost double from that of the baseline(+PDM). In the baseline(+PDM) it was mentioned as the most important need by 47% (33/70) of respondents and in this PDM1 it was mentioned by 79% (55/70). It is worth noting that the HHS of PDM1 was higher than that of the baseline(+PDM). Perhaps also schools being closed since the previous study has resulted in children no longer having access to school meal programs.

Food also remained as the most mentioned priority when considering the respondents top three priorities at 97% (68/70) of households, more than twice as often as the next item 'health and medication' with 43% (30/70). Psychological/mental health services was ranked the lowest among top

three needs in the baseline(+PDM) (1% - 1/70) but has increased to 6% (4/70) in the PDM1. Otherwise, most responses to the top one and top three remained fairly similar.





When asked what type of expenses the last cash transfer was used for (multiple selection), almost every respondent (97% - 68/70) reported using it towards the purchasing of food (the same percentage as in the baseline(+PDM)). Childcare and health & medication came in joint second with 73% (51/70).

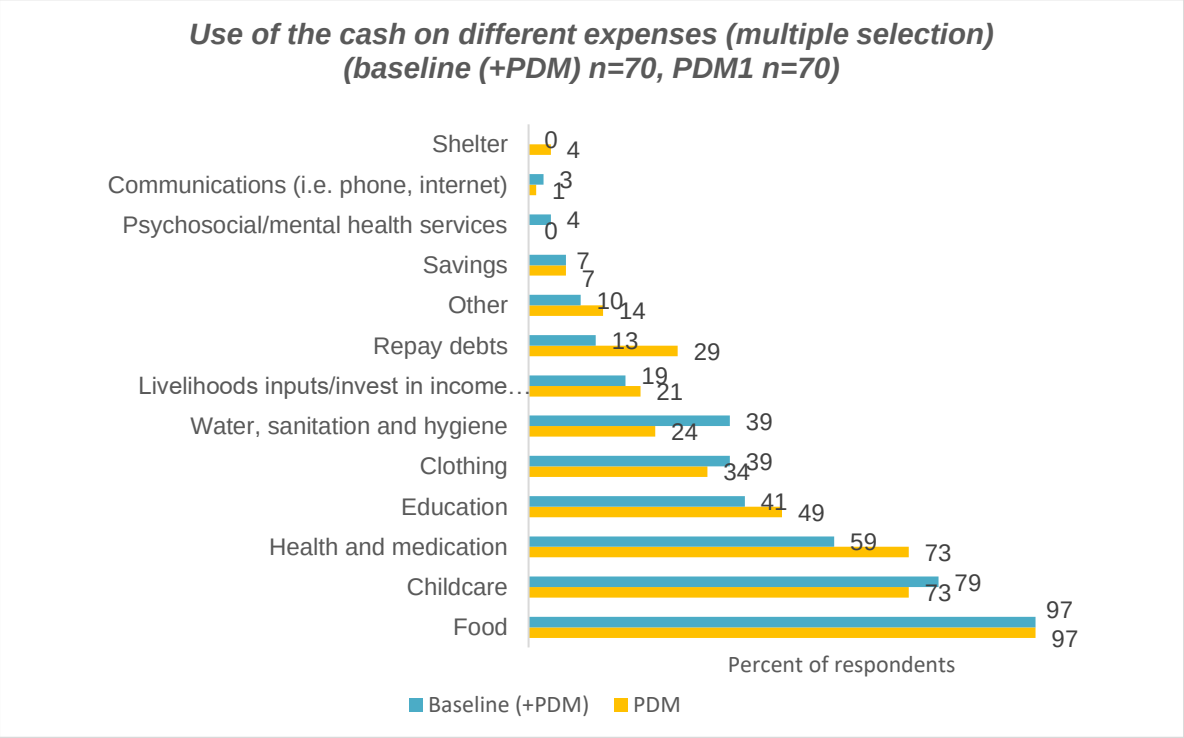
Childcare was slightly down from the baseline(+PDM) where it had been mentioned by 79% (55/70) and health & medication had increased quite a lot, up from 59% (41/70) in the baseline(+PDM).

There were substantial changes from the Baseline(+PDM) to the PDM1 among those who used the cash transfer on WASH, now down to 24% (17/70) from 39% (27/70) in the baseline(+PDM). Furthermore, those who reported repaying debts was up more than double to 29% (20/70) from 13% (9/70) in the baseline(+PDM). In comparison to the question on how much of their basic needs were met, 19/20 of those who reported repaying debts were only able to achieve 'some' of their basic needs, the other 1 person was able to achieve 'all'.

As in the baseline(+PDM), the results of this question are partially aligned with the respondent's ranking of what their priority needs of their household are, likely due in part to the economic prioritization sensitization sessions, which is explored further below.

As in the baseline(+PDM), two key differences between their priorities and their reported spending are that (1) childcare is much lower down on the reported priority needs of respondents (27% - 19/70 in PDM1) compared to the number of people who have spent their cash towards it (73% - 51/70 in PDM1) and (2) shelter is the third most needed priority (30% - 21/70 in PDM1), yet only 4% (3/70) mentioned using the cash transfer on it (up from 0% in the baseline(+PDM)).

With that noted, the chart below is only the percentage of respondents who have spent money towards each item, not how much they have spent on the item; therefore, it doesn't paint a full picture of the respondents needs. For instance, it may be that most families have spent money on childcare, but the amount of money spent is very low.



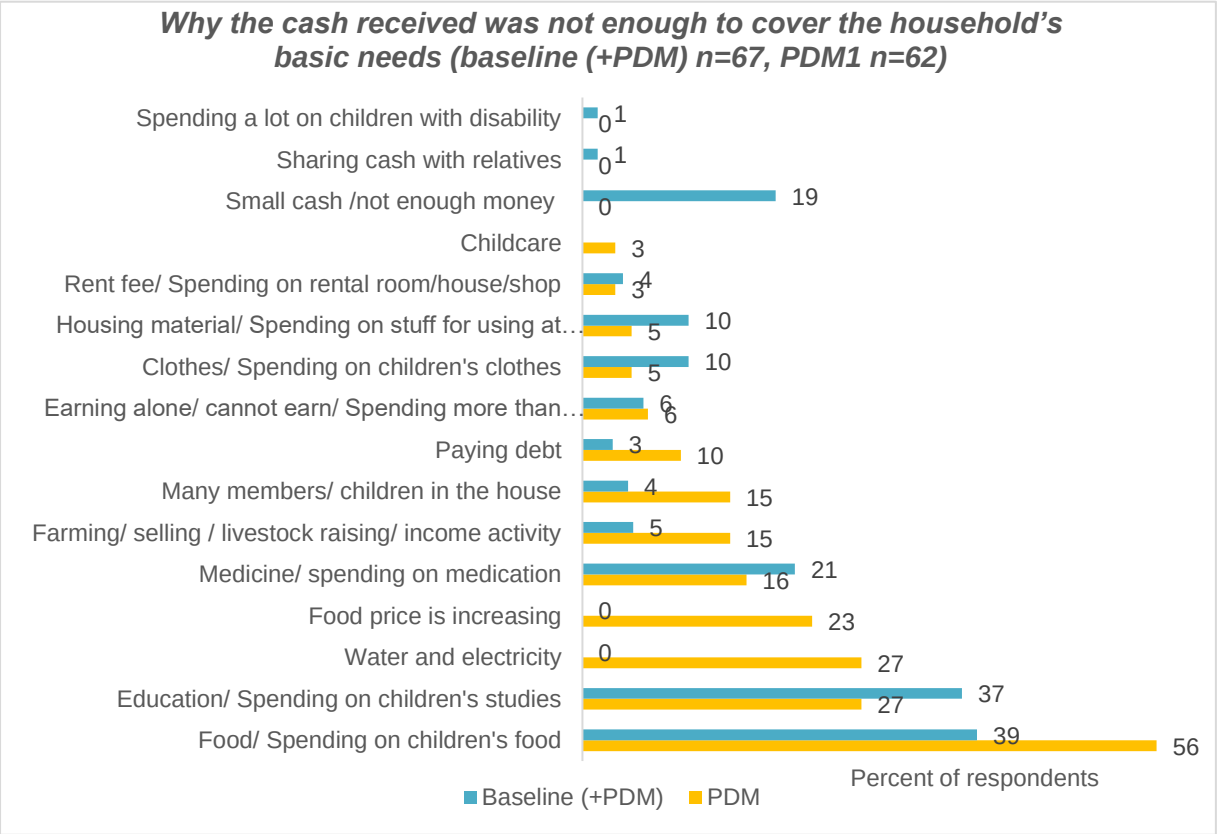
Although the cash transfers are not expected to cover the entirety of the households needs, the question as to why the households were unable to meet their needs was interpreted as referring to the cash transfers. The households typically mentioned that expenditures on their various needs exceeded that which the cash transfers provided them with. The question as to why the cash was not enough to cover the household's basic needs were insufficient was asked openly, with no answer codes provided. The

reasons for the cash transfer not covering the households needs vary quite significantly from the baseline(+PDM) and provide some interesting insights into changes affecting the respondents.

Less respondents reported spending on their children's education as a reason that they did not have enough to cover their basic needs, 37% (25/67) in the baseline(+PDM) to 27% (17/62) in the PDM1. However, more reported spending on their children's food as a reason, up to 56% (35/62) in the PDM1 compared to 39% (26/67) in the baseline(+PDM). Furthermore, 'food price is increasing' was mentioned by 23% (14/62), this had not been mentioned by any during the baseline(+PDM). These three factors are reflections on the changes in Cambodia since the baseline(+PDM) was conducted. Since the previous fieldwork, all schools nationwide have been closed and as such it stands to reason that spending on schooling is likely to decrease when it is remote or the costs categorized under another item (i.e. electricity). Furthermore, there are anecdotal reports of food costs increasing, in particular within the 'red zones' as travel bans and lockdowns have affected the country since the baseline(+PDM), possibly putting additional strain on these families' expenses. However, it is noted that at the time of writing the WFP does not see clear consolidated evidence of major food price increases within the country.

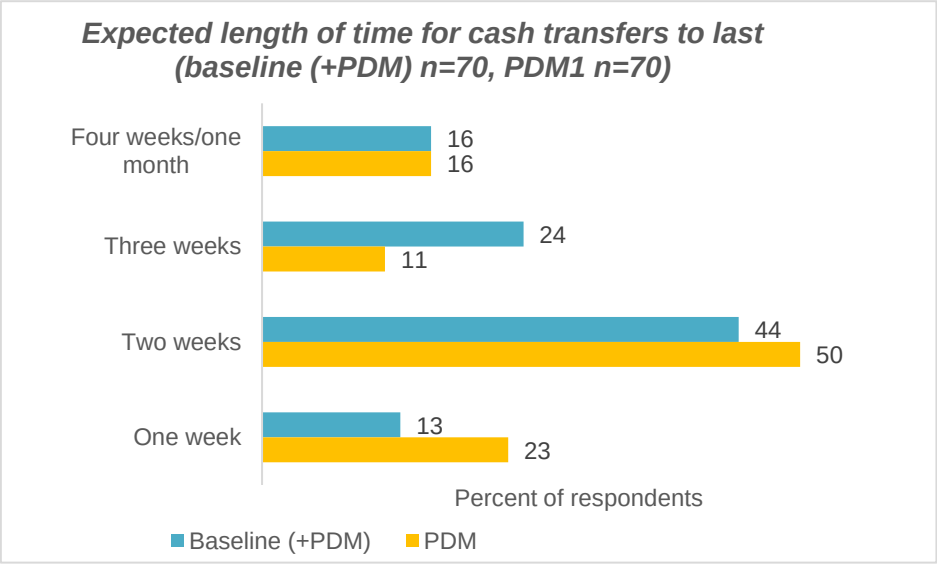
Another key change from the baseline(+PDM) was the mention of water and electricity. Previously it was not mentioned by any respondents in this question, but in this PDM1 it was, by 27% (17/62). It is probable that this is a result of respondents spending more time at home and thus increased spending on household utilities.

As in the previous questions, more respondents have reported repaying debts; here it was mentioned as a reason for the household to not cover all basic needs by 10% (6/62) up from 3% (2/67) in the baseline(+PDM).

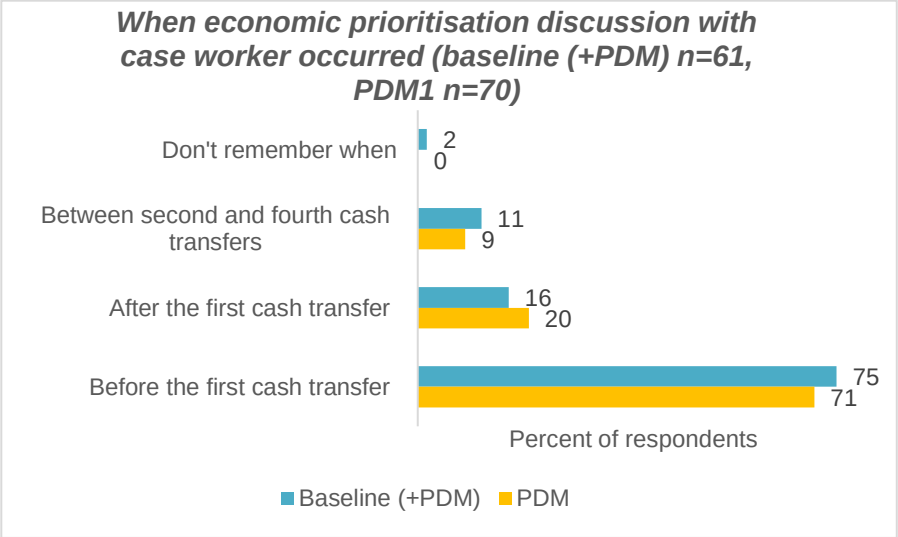


The length of time that the respondents think the cash transfers are supposed to help their household meet its basic needs has decreased since the baseline(+PDM). The percentage of respondents who

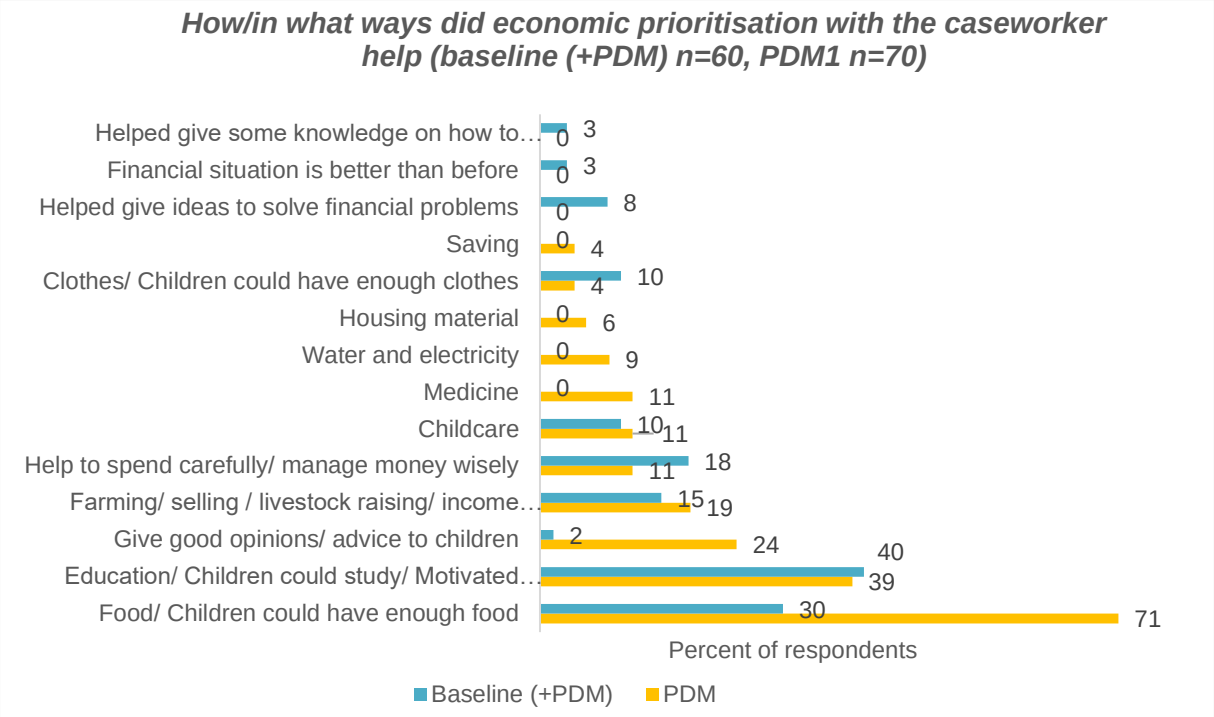
think it should help for one week has increased from 13% (9/70) in the baseline(+PDM) to 23% (16/70) in the PDM1 and the percentage who think that it should help for 2 weeks has increased from 44% (31/70) to 50% (35/70). In the baseline(+PDM) report, it was recommended that additional steps be taken to increase beneficiaries understanding regarding how to utilize the cash efficiently or to increase understanding on how long the money should help for. Due to the tight timeframe between the baseline(+PDM) report and this PDM1 data collection, it is likely that this recommendation was not widely implemented by the time of this PDM1 data collection. Following on from the baseline(+PDM), a cash program adaption webinar was held on 6th May 2021. One of the outcomes was that partners were to reach out to all households by phone 2 to 3 days before each cash transfer, to share key messages on the modalities and mechanism, transfer values and duration. This webinar was held after the data collection for the PDM1 and so results from this outcome are not reflected within these findings.



All the respondents (100% - 70/70) reported that their case worker talked to them about what their family/child's needs are, budgeting and how to prioritize household expenditure. This is an improvement as 87% (61/70) in the baseline(+PDM) reported that their case worker talked to them. The results for when the case workers spoke to the respondents was similar for this PDM1 as in the baseline(+PDM). 71% (50/70) of the respondents reported that they were talked to about this before their first cash transfer followed by 20% (14/70) after their first cash transfer and 9% (6/70) between the second and fourth transfers.



For the PDM1, all respondents agreed that these discussions helped them to better meet their family and children's needs. The talks primarily helped with methods for parents to take care of their children's food and education needs, as well as giving advice to their children. Furthermore, it helped them to spend or manage their money in line with their identified priorities. Compared to the baseline(+PDM), food was mentioned much more in PDM1 at 71% (50/70) compared to 30% (18/60).



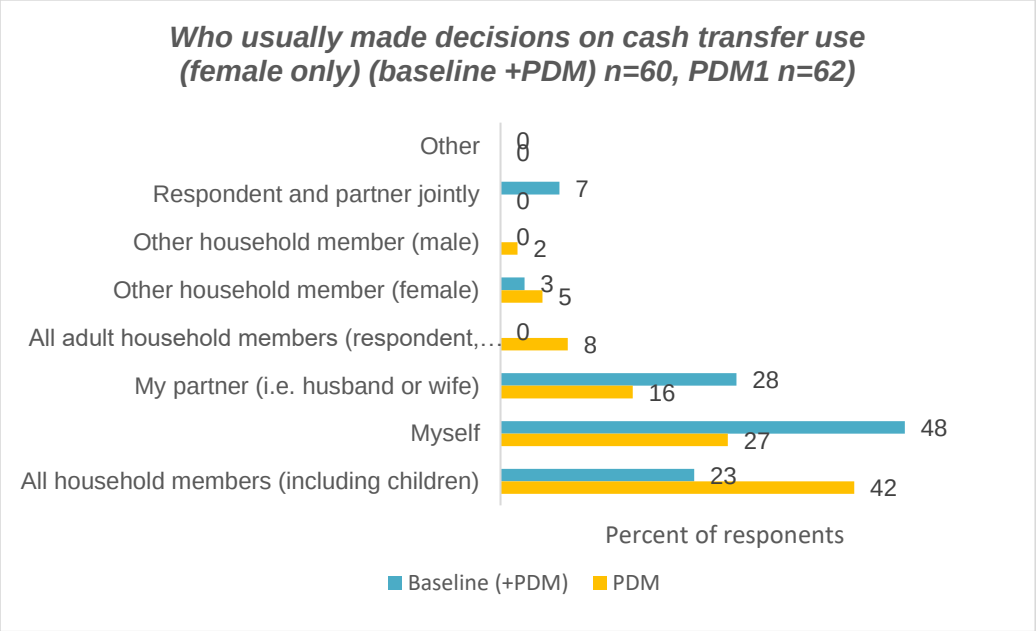
Finally, 81% (57/70) of the respondents reported that they have not shared the cash transfer with anyone outside their household, a decrease from 93% (65/70) in the baseline(+PDM). This may be worth following up on separately to understand why this is happening.

5.2. Percentage of women reporting shared decision-making on cash transfer use

Key indicator insight:

There is evidence that cash transfers increase women's decision-making power, alongside improvements in other empowerment indicators and reduced physical violence due to the cash transfers easing issues that drive these behaviors. Generally, women are reported as being involved in household decision-making if they fully or partially decide on the issue under question, in this PDM1 study the percentage of women reporting that they were involved remained the same as the baseline(+PDM) at 82% (51/62).

In this PDM1 89% (62/70) of respondents are female, in the baseline(+PDM) 86% (60/70) were female. Within the PDM1 results, usage of the cash transfer was decided less often only by the female respondent themselves (27% - 17/62) than in the baseline(+PDM) (48% - 29/60). It was decided by all household members, including children, more often in the PDM1 than in the baseline(+PDM) at 42% (26/62) compared to 23% (14/60).



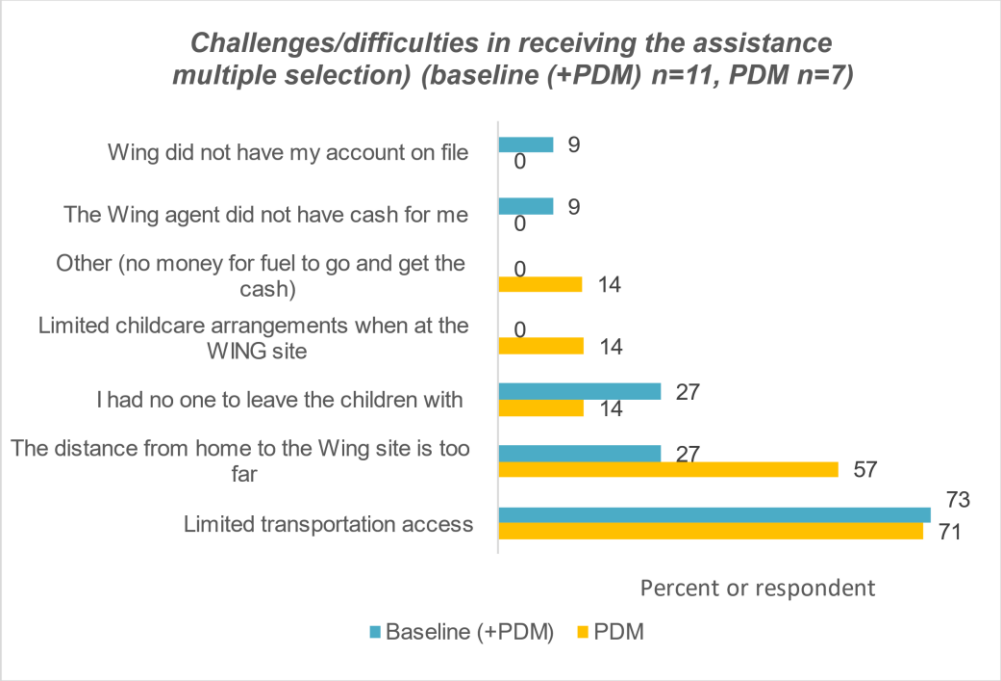
5.3. Percentage of households reporting that the assistance was delivered in a safe manner

Key indicator insight:

In line with humanitarian and development principles, it is vital that programs are delivered in a safe way, which is defined as assistance which prevents and minimizes as much as possible any unintended negative effects of the intervention, which can increase people's vulnerability to both physical and psychosocial risks. 100% (70/70) of respondents reported that they felt safe while receiving the cash transfer, an improvement on 96% (67/70) in the baseline(+PDM). Furthermore, 100% (70/70) reported that they had not heard of anyone else experiencing an unsafe situation, an improvement on 96% (67/70) in the baseline(+PDM).

All respondents were asked if there was anything SCI should be doing to ensure households are safely able to access the assistance provided. 99% (69/70) of respondents believed that there was not and only 1% (1/70) said that there was. That one person said that she wants the staff to deliver the money at home. By comparison, 13% (9/70) of respondents in the baseline(+PDM) study thought that there was something else SCI/partners could be doing to ensure households are safely able to access the cash.

However, 10% of households (7/70) in the PDM1 claimed that they had some challenges in receiving the assistance, although this is down from 16% (11/70) in the baseline(+PDM). As in the baseline(+PDM), the most often mentioned challenge was that the respondent had limited access to transportation 71% (5/7) followed by the distance to the Wing site being too far 57% (4/7). There was also one person (14%) who mentioned that there was limited childcare arrangements when at the Wing site and another who explained that they did not have money for fuel to travel to the Wing office. It has been reported that the FCF|REACT team and partners have been pro-active in seeking to address this issue by having WING agents reach out directly to households in remote locations. Although WING is currently deploying a food delivery system similar to the delivery apps NHAM 24 or Food Panda, it is not yet operational.



5.4. Percentage of households reporting that they received clear information about the assistance they would receive, including the amount

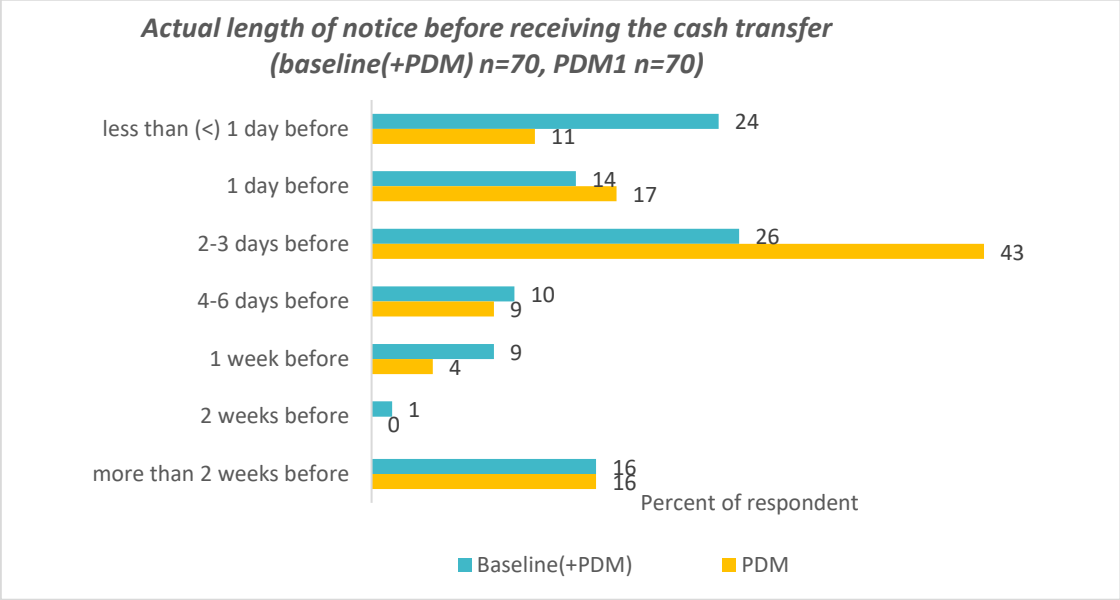
Key indicator insight:

In line with humanitarian and development principles, it is critical to ensure that communities are adequately informed of their entitlements and what to expect from SCI and partners. 87% (61/70) of the interviewed households reported that they received clear information about the assistance they would receive, including the amount. A substantial improvement from the baseline(+PDM), which was 54% (38/70).

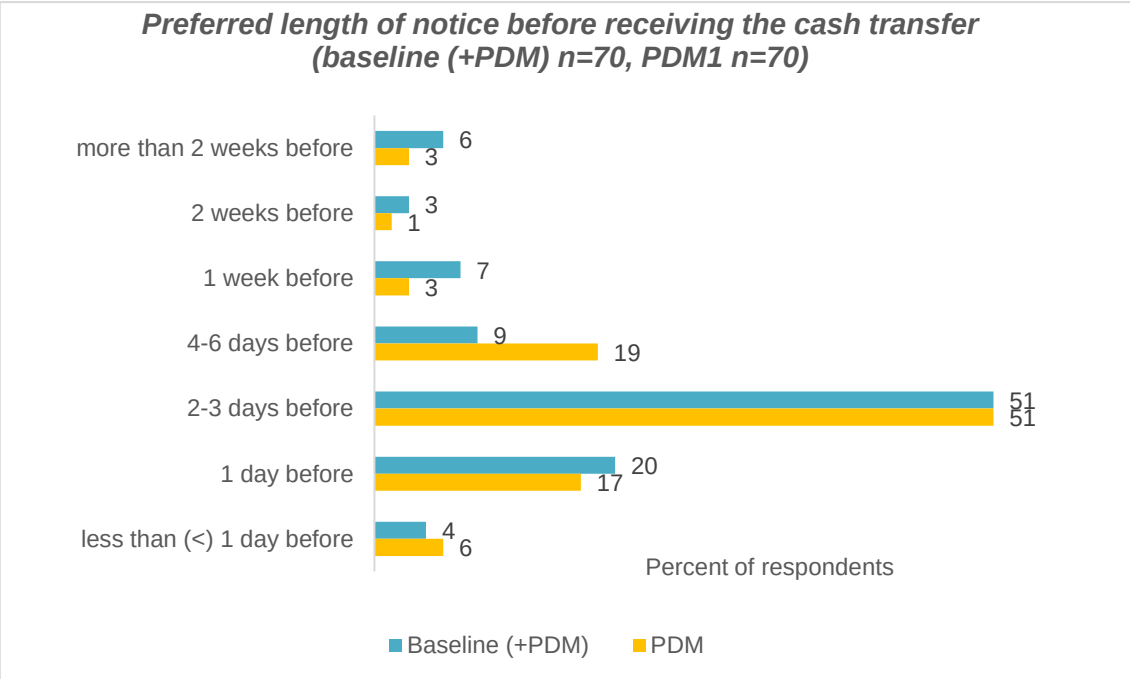
87% (61/70) of households reported understanding why households were selected to receive the cash transfer from SCI/partner. This is a substantial improvement from the baseline(+PDM) where 64% (45/70) understood why they were selected. All 100% (70/70) of respondents reported that they received the same amount of money that they were told they would receive, up from 87% (61/70) in the baseline(+PDM). However, it is expected that the lower figure found during the baseline(+PDM) was due to WING mistakenly charging a fee for withdrawal rather than a program or technical issue. WING is expected to reimburse those fees to households in May. 100% (70/70) in the PDM1 reported that they received notice of the cash transfer clearly and far enough in advance, up from 89% (62/70) in the baseline(+PDM).

29% (20/70)[1] of the respondents were informed about the date of the cash transfer one day or less than one day before the delivery date with 11% (8/70) being less than one day before. 43% (30/70) received notice at the most requested time of 2-3 days before. This is an improvement on the baseline(+PDM) where 24% (17/70) received notice less than 1 day before.

[1] The graph shows 11.43% (8/70) for less than 1 day before and 17.14% (12/70) for 1 day before, due to rounding this is 29% (20/70), although the graph appears to add up to 28%.



There is consistency between the baseline(+PDM) and the PDM1 that the most preferred notice to be informed about the delivery date of the cash transfer is within 2-3 days before as it had 51% (36/70) in both the baseline(+PDM) and PDM1. As part of the program adaptation co-defined by partners and FCF|REACT team discussed in the 6th May webinar, social workers are to reach out by phone to each of the households 2 to 3 days before each cash transfer, however, this will be implemented before the next cash transfer and is not reflected within the PDM1 results.

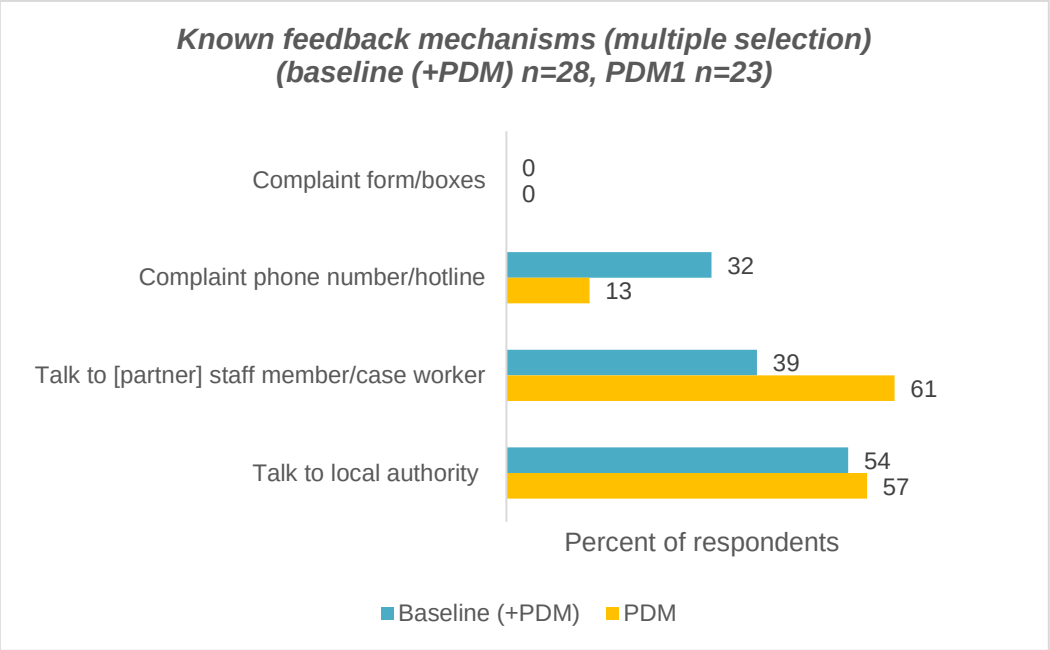


5.5. Percentage of households who can identify at least one available channel for providing complaints or feedback about the assistance

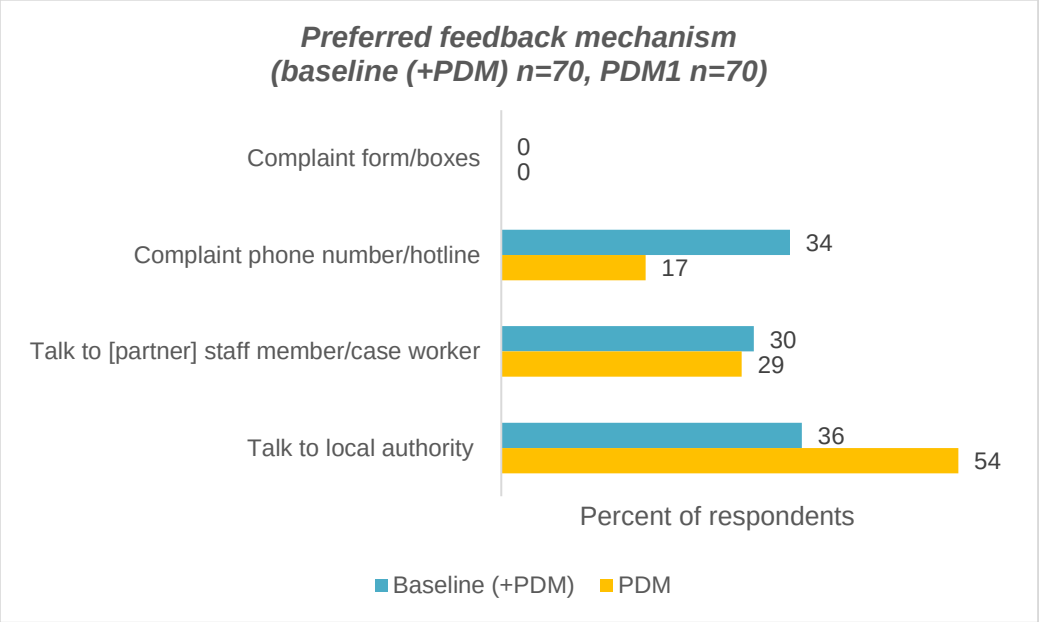
Key indicator insight:

In line with humanitarian and development standards and principles, it is vital to ensure that communities have access to safe and responsive mechanisms to handle feedback, concerns and complaints. If a household reports being aware of a channel for providing complaints or feedback about the assistance, and can then identify at least one available channel, they will be counted towards this indicator. Overall, only 33% (23/70) of households could identify at least one available channel for providing complaints or feedback about the assistance, slightly lower than the 40% (28/70) achieved in the baseline(+PDM).

Of those who knew where to submit a complaint, 61% (14/23) knew of a partner staff member/case worker, 57% (13/23) identified that they could talk to a local authority, and 13% (3/23) were aware of a complaint phone number/hotline. A higher percentage of those who knew where to submit a complaint reported this as being through talking to a partner staff member/case worker in the PDM1 (61% - 14/23) than in the baseline(+PDM) (39% - 11/28) and a lower percentage mentioned the complaint phone number/hotline. As in the baseline(+PDM) none of the respondents identified complaint form/boxes, which is to be expected as few partners are relying on this mechanism.



All respondents were presented with the different available options for complaints and asked which they would feel most comfortable using. Unlike in the baseline(+PDM) where preference was almost equally split, there is a preference among the respondents in the PDM1 to speak to a local authority (54% - 38/70), then staff member/case worker (29% - 20/70) then complaint phone number/hotline (17% - 12/70). As in the baseline(+PDM), complaint form/boxes received no votes. It is unclear why this change in preference might have occurred between baseline(+PDM) and PDM1 and may simply be due to chance or the preference of the individual respondents.



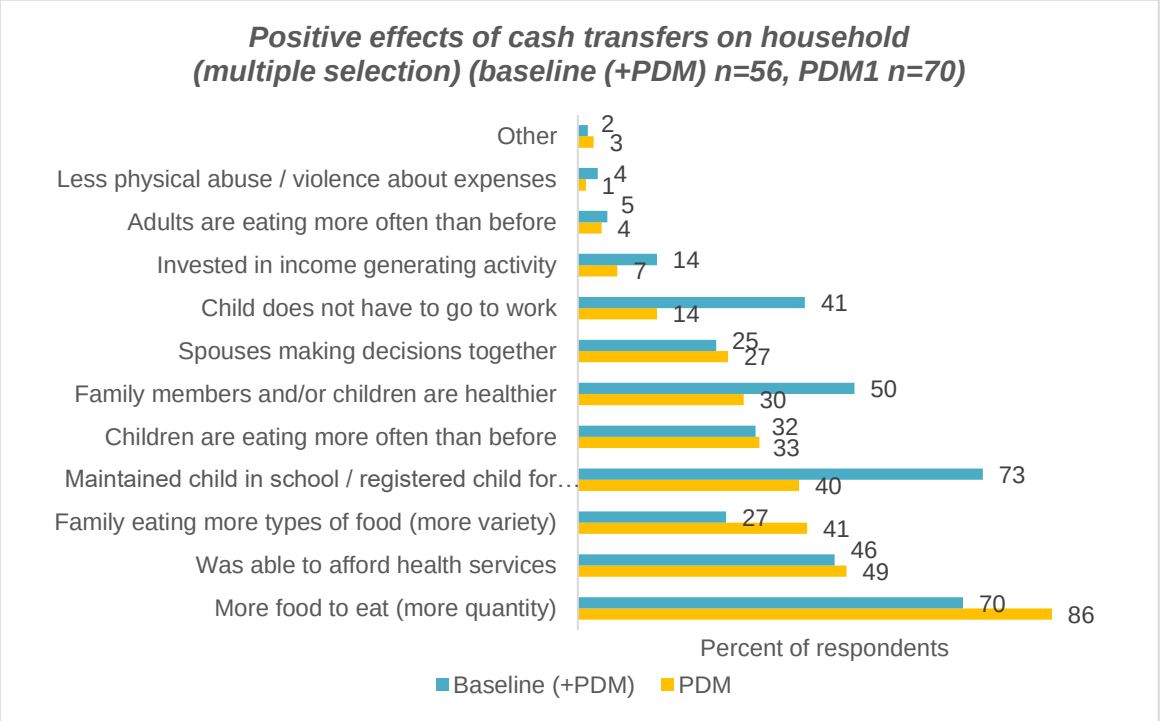
17% (12/70) of respondents reported that they had or knew someone who had placed a complaint with a partner/Save the Children, this is a substantial increase on the baseline(+PDM) result of 6% (4/70). Of the 12 respondents who had or knew someone who had placed a complaint, only one person (8%) claimed that the complaint was not resolved.

5.6 Positive and Negative Effects of Cash Transfers on Households

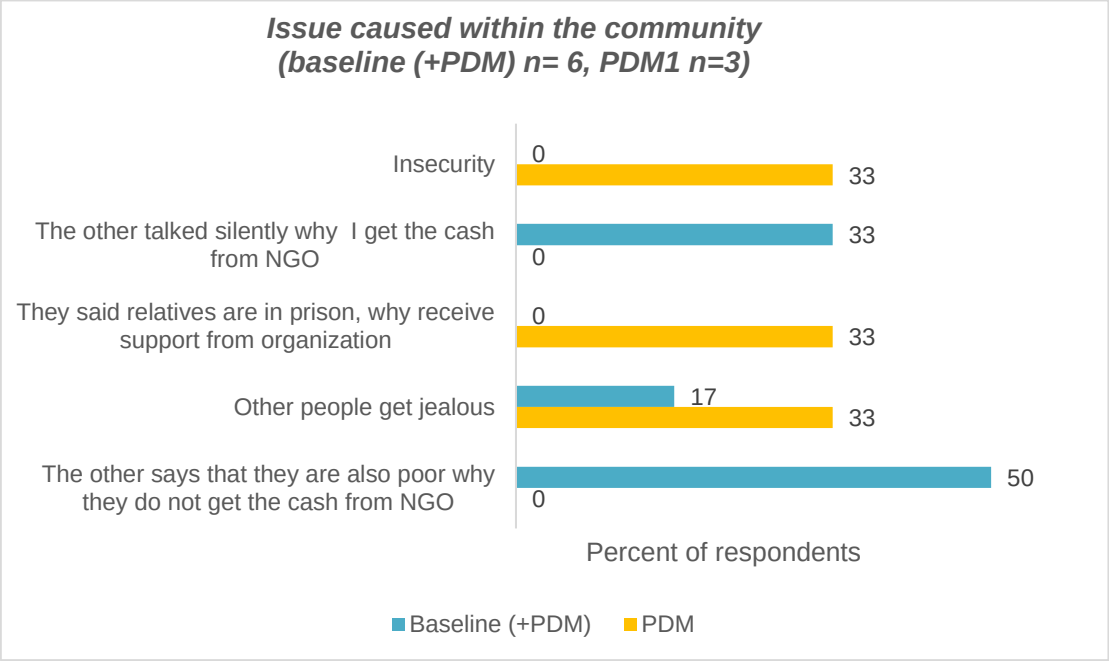
Key indicator insight:

In total, 100% (70/70) of the respondents reported that the cash transfers had a positive effect on their household or relationship within them, up from 80% (56/70) in the baseline(+PDM). 86% (60/70) of respondents noted that the cash had enabled them to have more food to eat (up from 70% - 39/56). None of the respondents identified that there had been any negative effects (down from 1% -1/70) in the baseline(+PDM).

A possible negative sign, and likely due to the on-going effects of the wide COVID-19 outbreak, the number of respondents reporting that a positive effect of the cash transfer on the household had been that their child does not have to go to work reduced from 41% (23/56) in the baseline(+PDM) to 14% (10/70) in the PDM1. Furthermore, the number of respondents who reported that a positive effect of the cash transfer had been to maintain their child in school reduced from 73% (41/56) to 40% (28/70).



4% (3/70) of respondents noted that the cash transfers had caused issues within their communities, this was down from 9% (6/70) in the baseline(+PDM). The responses were split equally among three different issues and were largely different to those of the baseline(+PDM).



6. Recommendations for future implementation

1. **More households reported sharing their cash with people outside of their households.**

From the results of this study, it is unclear why these respondents are sharing the money with others. Due to these unprecedented times during a pandemic, it may be that they are helping others in their family or community who are in desperate situations. Unfortunately, at the moment we do not know why they are sharing with others and possible implications of this. It is recommended that case workers follow up with these respondents or that a question is added to the next set of data collection to understand this further.

2. **Greater understanding on the effects of COVID-19 on education is needed.**

Although it is not asked directly as part of the PDM1 data collection, it stands to reason based on the national closure of schools and decreased number of respondents mentioning expenditure on education that children's education may have been negatively affected by the latest outbreak of COVID-19. It was outside of the scope of this PDM1 study, but it would be interesting to understand further about the effects of COVID-19 on children's education. In the PDM2 study to be conducted questions regarding attendance are covered (as they were in the baseline(+PDM), yet, more qualitative understanding regarding whether families are capable of enabling remote learning for their children, if they organise any alternative learning and how effective any of this different way of learning is, would be interesting to understand. Understanding could possibly be achieved through a selection of additional questions added to PDM2, social workers gathering information through case management/money matter sessions or as part of other SCI or partner projects.

3. **Understanding effects of current COVID-19 community outbreak on children entering the workforce.**

The number of respondents that mentioned that a positive effect of the cash transfers is that children didn't need to go to work reduced from 41% to 14%. This is not clear evidence that their children have in fact started working, but with school closures, it is a concern. The PDM2 study asks questions directly related to children's work and will provide clearer evidence if this is, or is not, occurring. However, in the meantime, it would be proactive for social workers to begin discussing this with families. As such, SCI and partner organisations should raise awareness to social workers of this possible issue.

4. **Understanding why respondents have increased paying off debts.**

Many more households have reported paying off debts than in the baseline(+PDM) previously. Currently, there is little evidence to point to the underlying reasons as to why debts are being paid off and so the rationale may be either positive (e.g., increased income) or negative (e.g., coercion from debt collectors). As the reasoning may be sensitive and as it is difficult to ask succinctly within a questionnaire, it is recommended that further understanding of this topic is not added to the PDM2. Instead, it is recommended that case workers are familiarized with this issue and may follow up with beneficiaries during their next visitation.

7. Annexes

7.1. Evaluation Framework

	Indicator	Monitoring Activities for Collection	Baseline	PDM 1
1	% of households reporting child separation from caregiver (including because of work-related migration) because of lack of money.	Baseline (+PDM); PDM 2; End line	**Before the cash transfer** 21% of households (15/70) reported child separation from caregiver due to lack of money. **Since the cash transfer** 14% of households (10/70) reported child separation from caregiver due to lack of money.	
2	% of households that reported having had children go to work during the past 30 days either regularly (on a weekly basis) or frequently (more than once a week), in order to meet basic needs.	Baseline (+PDM); PDM 2; End line	10% (7/70) of households reported having had children go to work during the past 30 days either regularly (on a weekly basis) or frequently (more than once a week), in order to meet basic needs.	
3	% of children who were accessing basic education before COVID and are now no longer doing so.	Baseline (+PDM); PDM 2; End line	11% (13/118) of children who were accessing basic education before COVID are no longer doing so.	
4	% of households that report feelings of stress and anxiety	Baseline (+PDM); PDM 2; End line	36% (25/70) of households reported that in the past month there has been more stress and tension in the home and/or household members feel worried or anxious and/or there has been some violence in the home.	
5	% of households who report being able to meet the basic needs of their households (all/most/some/none) according to their priorities	Baseline (+PDM); PDM 1; PDM 2; End line	4% (3/70) of households reported that they can fulfil all their basic needs, 27% (19/70) can fulfil most of their basic needs while 69% (48/70) can still only fulfil some of them.	11% (8/70) of households reported that they can fulfil all their basic needs, 20% (14/70) can fulfil most of their basic needs whilst 69% (48/70) can still only fulfil some of them.
6	Average Reduced Coping Strategies Index (rCSI) Score	PDM 2; End line		
7	Prevalence of households with moderate or severe Household Hunger Scale (HHS) score	PDM 2; End line		

8	% of households who report access to safe water for drinking, cooking and cleaning	Baseline (+PDM); PDM 2	80% (56/70) of household's report access to safe water for drinking, cooking and cleaning.	
9	% of women reporting shared decision-making on cash transfer use	Baseline (+PDM); PDM 1; PDM 2	82% (49/60) of women reported shared decision-making on cash transfer use.	82% (51/62) of women reported shared decision-making on cash transfer use.
10	% of households reporting that the assistance was delivered in a safe manner	Baseline (+PDM); PDM 1; PDM 2	96% (67/70) of respondents reported that the assistance was delivered in a safe manner.	100% (70/70) reported that the assistance was delivered in a safe manner.
11	% of households reporting that they received clear information about the assistance they would receive, including the amount	Baseline (+PDM); PDM 1; PDM 2	54% (38/70) of households reported that they received clear information about the assistance they would receive, including the amount.	87% (61/70) of households reported that they received clear information about the assistance they would receive, including the amount.
12	% of households who can identify at least one available channel for providing complaints or feedback about the assistance	Baseline (+PDM); PDM 1; PDM 2	40% (28/70) of households could identify at least one available channel for providing complaints or feedback about the assistance.	33% (23/70) of households could identify at least one available channel for providing complaints or feedback about the assistance.
13	Average percent change in earnings following participation in USG-assisted workforce development programs	Baseline (+PDM); End line	The average money earned by those who work is $104.29 per month (54 of 70 respondents)	
14	Percent of individuals with new employment following participation in USG-assisted workforce development programs	Baseline (+PDM); End line	77% (54/70) of respondents worked in the previous month to earn money.	
15	Percent of individuals with better employment following participation in USG-assisted workforce development programs	End line		
16	Number of individuals in the agriculture system who have applied improved management practices or technologies with USG assistance	End line		

7.2 Research Materials and Tools

Research materials and tools were developed in English by SCI utilizing a number of different existing tools to be able to report against the indicators identified above. Tools utilized in this development included:

- Alliance for Child Protection in Humanitarian Action CVA and Child Protection Tool 2 and Tool 3
- Save the Children International Cash Post-Distribution Monitoring tool

- Save the Children US Emergency Food Security and Livelihoods Post-Distribution Monitoring template
- ECHO Protection Mainstreaming Indicator Questionnaire
- USAID Measuring Employment and Earnings using the Workforce Outcomes Reporting Questionnaire (WORQ)

The tools were submitted to the Save the Children US Ethics Review Committee for approval (SCUS-ERC-FY2021-25). Full materials included in annexes 6.3 Baseline +PDM, 6.4 PDM1 and 6.5 Last PDM below.

7.3 References

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7.4 PDM1 research tool

FCF|REACT Cash & Livelihoods

Post-Distribution Monitoring Tool

[Note: the numbering of questions in this tool matches the question number in the baseline/PDM tool which is why some numbering is missing as it is not asked in this PDM]

Introduction and Consent

"Hello. I am [*insert name*]. I work for [*insert name of your company*] and today I am here on behalf of Save the Children to conduct a survey with randomly selected households who are participating in our cash transfer and livelihoods program. The questions will take about 20-30 minutes to complete".

"I would like to ask you some questions about your experience receiving cash transfer support from Save the Children and [*name of partner*]. The questions focus on safety and child protection benefits related to the cash transfer and will help Save the Children and *[name of partner*] improve the program and the safety of those involved as a part of child protection case management."

"Your participation is voluntary. If you agree to participate, you can choose to skip any and/or all questions or choose to end the survey at any time. Whether you participate or not, or your answers, will not affect or change the level or frequency of cash you or anyone else gets in the future."

"If you do not understand a question, please ask me to explain it. Your answers are confidential. That means that I may share details of what you have told me so that we can improve our work, but I will not tell anyone who told me these things. However, if you tell me that you know a child who has been harmed, and continues to be at risk, I would have to tell Save the Children so we could get help for the child, and so we can find ways to prevent that child or other children from being harmed again."

"Do you have any questions for me or concerns you want to discuss with me, before we begin?"

"If you have any concerns you want to share after this interview, or there are any issues with the way we carried out this interview, you may contact the following person [*insert details for company*].

"Do you agree and consent to be interviewed?"

- Yes
- No – if they say no, thank them for their time and terminate the interview.

Client Details

Household ID number: [see HES]

1. Is the respondent the same as for the HES?
   - Yes
   - No

   1a. Respondent Name:
   1b. Respondent Gender:
   1c. Respondent Age:
   1d. Respondent Disability Status:

Cash Transfer Utilization, Information Basic Needs & Decision-Making

2. Do you understand why households were selected to receive the cash transfer from Save the Children/Partner?
   - Yes
   - No

3. What are the 3 priority needs of the household (in order of importance)

   - Food
   - Clothing
   - Shelter
   - Health and medication
   - Water, sanitation and hygiene
   - Education
   - Child care
   - Repay debts
   - Savings
   - Livelihoods inputs
   - Communications (i.e. phone, internet)
   - Psychosocial/mental health services
   - Legal/justice services
   - Other, specify

4. How much cash in USD/Reil did you receive from Save the Children/Partner in the last distribution? *[Note: cash can be received in either USD or Reil so the currency will also need to be noted with the response]*

5. Was the amount you received equal to the amount you were told you would receive?
   - Yes (go to question 7)
   - No
   - Don't know (go to question 7)

6. If no, was it more or less than you were told you would receive?
   - Less
   - More

6a. Do you know why there was a difference between what you were told you would receive and what you actually received?

7. Was the date of the cash transfer communicated to you clearly and far enough in advance?
   - Yes
   - No

8. How long before the cash transfer were you informed about the date of the cash/voucher distribution?
   - less than (<) 1 day before
   - 1 day before
   - 2-3 days before
   - 4-6 days before

- 1 week before
- 2 weeks before
- more than 2 weeks before

9. How far ahead of time would you ideally like to be notified about the date of the next cash/voucher distribution?

- less than (<) 1 day before
- 1 day before
- 2-3 days before
- 4-6 days before
- 1 week before
- 2 weeks before
- more than 2 weeks before

10. For how long do you think one cash transfer is supposed to help your household meet its basic needs?

- One week
- Two weeks
- Three weeks
- Four weeks/one month
- More than four weeks/one month

11. Is your household able to meet all, most, some, or none of its basic needs, as you define them [in the last month]?

- All (go to question 13)
- Most
- Some
- None

12. Why did it not cover the basic needs of the household over the [last month]?

13. Do you think cash/voucher assistance is the most effective form of assistance to help you and your family to meet your basic needs?

- Yes (go to question 15)
- No
- Don't know (go to question 15)

14. Why not? And what would be more effective?

15. You mentioned that this Save the Children program gave you ${*amount received*} for the last cash transfer. Can you please tell me which type of your expenses this money went to? *[Multiple selection]*:

- Food
- Clothing
- Shelter
- Health and medication
- Water, sanitation and hygiene
- Education

- Child care
- Repay debts
- Savings
- Livelihoods inputs/invest in income generating activity
- Communications (i.e. phone, internet)
- Psychosocial/mental health services
- Legal/justice services
- Other (please specify)

16. Who usually makes decisions about how the cash transfer is used?
- Myself
- My partner (i.e. husband or wife)
- Respondent and partner jointly
- Other household member (female)
- Other household member (male)
- Respondent and other household member jointly
- All adult household members (respondent, partner and others)
- All household members (including children)
- Other (specify)

16a. Did your case worker talk to you about what your family/child's needs are, budgeting and how to prioritize household expenditure?

- Yes
- No (go to question 17)

16b. When did they talk to you about this? [Multiple selection]

- Before the first cash transfer *(prompt: first cash transfer was end of December)*
- After the first cash transfer *(prompt: first cash transfer was end of December)*
- Between second and fourth cash transfers *(prompt: second end of January, fourth will be end of March)*
- Don't remember when

16c. Did these discussions help you to better meet your family and children's needs?

- Yes
- No (go to question 16e)

16d. How/in what ways did it help? (go to question 17)

16e. Why did it not help and how could it be improved?

17. Did you share any of the cash with people not in your household?
- Yes
- No

Positive & Negative Effects on Household

20. Have the cash/vouchers had any positive effect on your household or relationships within your household?

- Yes
- No (go to question 22)

21. How has receiving the cash/vouchers positively affected your household? *[multiple selection]*

- Less physical abuse / violence about expenses
- Less verbal abuse/threats about expenses
- Spouses making decisions together
- Invested in income generating activity
- Postponed child's marriage
- Child does not have to go to work
- Maintained child in school / registered child for school
- Was able to afford health services
- Family members and/or children are healthier
- More food to eat (more quantity)
- Family eating more types of food (more variety)
- Children are eating more often than before
- Adults are eating more often than before
- Other (please specify)

22. Have the cash/vouchers had any negative effect on your household or the relationships within your household?

- Yes
- No (go to question 24)

23. How has receiving the cash/vouchers negatively affected your household relationships? [NOTE: PLEASE DO NOT READ OPTIONS OUT LOUD] *[multiple selection]*

- Physical abuse / violence about how to spend the cash
- Threat of physical abuse/violence about how to spend the cash
- Verbal abuse about how to spend the cash
- Tensions about who received or how to spend the cash
- Forced withholding of cash
- Husbands or partners demanding sex without consent
- Other (specify)

24. Has this program caused tensions or problems within the community?

- Yes
- No (go to question 26)

25. If you feel comfortable, please tell me what types of problems it has caused.

Safety Receiving Cash Transfer

26. Did you feel safe while receiving the last cash transfer?

This includes safety while going to receive assistance, waiting for assistance and coming back home after assistance

- Yes (go to question 32)
- No

27. If you are comfortable telling me, what type of unsafe situation did you experience in receiving the assistance *[multiple selection]*?

- Theft of money received
- Physical assault
- Verbal assault
- Sexual assault
- Attempted assault
- Lack of respect/consideration
- Fighting/problems in household over assistance
- Safety issues due to distance/transportation
- Safety issues due to water and sanitation facilities (or lack thereof)
- Other (please specify)
- *Don't feel comfortable answering*

28. Have you already reported this to anyone?

- Yes
- No (go to question 30)

29. Who did you report it to?

30. Would you like a Save the Children staff member to follow-up with you regarding this unsafe situation

- Yes
- No (go to question 32)

31. What is the best and safest way to contact you?

32. Did you hear of anyone else experiencing an unsafe situation or violence as a result of receiving the assistance?

- Yes
- No (go to question 34)

33. If you are comfortable telling me, what type of unsafe situation did you experience in receiving the assistance *[multiple selection]*?

- Theft of money received
- Physical assault
- Verbal assault
- Sexual assault
- Attempted assault
- Lack of respect/consideration
- Fighting/problems in household over assistance
- Safety issues due to distance/transportation

- Safety issues due to water and sanitation facilities (or lack thereof)
- Other (please specify)
- Don't feel comfortable answering

34. Is there anything you think we should be doing to ensure households are safely able to access the assistance/cash transfer?
- Yes
- No (go to question 36)

35. Please detail.

36. Were there any challenges or difficulties you had in receiving the assistance?
- Yes
- No (go to question 38)

37. What were the challenges/difficulties you experienced in receiving the assistance *[multiple selection]*?
- No time left to work on other duties
- Limited transportation access
- The Wing agent did not have cash for me
- Wing did not have my account on file
- I had no one to leave the children with
- Limited childcare arrangements when at the WING site
- Road blockages
- The distance from home to the Wing site is too far
- There are no water and sanitation facilities at the Wing site
- Other (please specify)

Feedback & Complaints Mechanism

38. Are you aware of any complaint or feedback mechanisms where you could submit a concern if you needed to?
- Yes
- No (go to question 40)

39. What feedback mechanisms do you know about *[multiple selection]*?
- Complaint phone number/hotline
- Complaint form/boxes
- Talk to [*partner*] staff member/case worker
- Talk to local authority (i.e. village chief, CCWC, commune chief)

40. Our program has the following complaint mechanisms in place [Enumerator note: Please read the list.] If you had a complaint, which of these mechanisms would you feel most comfortable using?
- Complaint phone number/hotline
- Complaint form/boxes
- Talk to [partner] staff member/case worker
- Talk to local authority (i.e. village chief, CCWC, commune chief)

41. Have you or somebody you know placed a complaint with *Partner*/Save the Children?
    - Yes
    - No [go to close]

42. Was the complaint resolved?
    - Yes
    - No
    - Don't know

Closing

"Thank you for your time today. Your answers will help us understand your experience participating in the program and help improve the support we provide.

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